(1)

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[redacted] Master [redacted] cherry - 605 014.

2024062570844

Patient's Name [redacted] PIMS No 2024062570844

Date & Time	Admission Notes c/s/B [redacted]
10/3/26 12:00PM	A 6½ year old male child came with c/o Fever (3/3/26) x 7 days. c/o vomiting x 6 days non projectile non bilious. c/o loose stools x 5 days. c/o cough & Rhinorrhoea x 7 days c/o poor oral intake. c/o decrease urine output. No c/o burning micturition. c/o Irritability. No c/o altered sensorium. No c/o fast breathing.

Past history :- Nil significant.

O/E ⇒ child was active & alert

HR – 114/min
RR – 22/min
SpO2 – 98%

C/E ⇒ : eschar (R) Iliac region.

R/S ⇒ B/L air entry NVBS.
CVS ⇒ S1S2
CNS ⇒ NFND
P/A ⇒ Soft non tender Liver spam 1cm.

Δ Fever under evaluation Plan
: Enteric fever ⇒ To send CBC, CRP, blood culture, widal, urine
: Scrub ⇒ To send LFT, RFT, Scrub serotest
⇒ To send H1N1 swab.

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2

2024062570844

Patient's Name ..... PIMS No .....

Date & Time	
11/3/26	C/S/B [redacted]
DOT-8	Δ - Tropical Fever. Probable Enteric fever ? Scrub typhus.
	⇒ c/o fever x 3 episodes high grade.
	⇒ c/o ↓ oral intake
	⇒ No c/o lethargy
Based on history & examination ⇓ [No features of complication]	⇒ No c/o Irritability
	⇒ c/o Vomiting x1 episode.
	⇒ c/o loose stools x 3 episodes
[u/o ⇒ 1.2ml/kg/day]	⇒ No c/o altered Sensorium
	⇒ No c/o bone pain
	⇒ No c/o ↓ u/o
	⇒ No c/o abnormal movements.
	⇒ No c/o abdominal pain
O/E ⇒	HR - 132/min [Temp - 102.4°F] Spleen tip is palpable
	RR - 34/min
	SpO2 - 99% CP/PP +++/++
S/E ⇒ CVS ⇒ S1S2	P/A ⇒ soft non tender
CNS ⇒ NAND	Liver is palpable 1cm. Spleen 1cm

RCM

[redacted]

Master [redacted] ESS SHEET

Patient's Name .....2024062570844..... PIMS No .....

Date & Time	
	Plan ⇒ To continue IV ceftriaxone ⇒ To continue antipyretics ⇒ To continue oral doxycycline ⇒ To continue IV fluids. ⇒ To trace blood reports widal serum, dengue reports.
11/03/26 10:00AM	C/S/B - [redacted]
	Child Reviewed, C/o fever C/o Vomiting C/o Loose stools No c/o clothing No c/o irritability No c/o Altered Sensorium

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Master [redacted]

2024062570844

SS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
12/03/26 7:00 AM	C/S/B - [redacted]

Δ = ENTERIC FEVER

⇒ c/o fever at 1:30 AM (104°F)
⇒ No c/o abdominal pain
⇒ No c/o ↓ oral intake
⇒ No c/o lethargy
⇒ No c/o poor oral intake.
⇒ No c/o Irritability
⇒ No c/o Vomiting.
⇒ No c/o lethargy
⇒ No c/o headache & abnormal movements
⇒ No c/o reduced urine output.

O/E ⇒ child was active & alert
HR - 96/min
RR - 28/min
SpO2 - 97%.
BP - 100/60 mmHg.

S/E :- CVS ⇒ S1S2
CNS ⇒ NFND
P/A ⇒ Soft nontender Liver is palpable 3cm below RCM 10.5 cm. Liver span.
R/S ⇒ B/L air entry NVBS

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PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
12/3/26 6:00 am	C/S/B - [redacted])
	Δ = ENTERIC FEVER
	No new Complaints,
	O/E: Alert, Active, Afebrile
	PR: 100/min RR: 22/min SpO2: 98% @ Room Air
	S/E:- RS: B/L AE (+) P/A: Soft, non tender, Liver palpable 4cm Below RCM Liver Span = 10.5 cm CVS: S1 S2 (+)
[redacted]	Plan:- 1) To Continue Ceftriaxone 2) W/F Fever Spikes 3) W/F Complications 4) If Worsening present — to Add Azithromycin.

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[redacted]

Master [redacted]

PROGRESS SHEET

2024062570844

Patient's Name ... PIMS No ...

Date & Time

13/03/26 1:00pm

C/S/13 - [redacted]

Δ - ENTERIC FEVER
Nutritional anemia.
⇒ c/o fever
⇒ c/o loose stools x 2 episodes.
⇒ No c/o lethargy
⇒ No c/o Irritability
⇒ No c/o altered sensorium
⇒ No c/o abdominal pain
⇒ No c/o decrease urine output.
⇒ No c/o altered behaviour
⇒ No c/o Joint pain

O/E: child wns
HR: 100/min
RR: 26/min
SpO2: 98%
BP: 100/70mmHg

S/E:-
CVS: S1 S2 (+)
CNS: No focal neurological deficit
P/A: Soft non tender
Liver is palpable 10.5 cm liver span
⇓
4cm RCM

R/S ⇒ B/L air entry NVBS.

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
	Plan => To add azithromycin => To continue ceftriaxone => To continue antipyretics => w/f worsening.
13/3/26 10:00 AM	C/S/B – [redacted] Δ = ENTERIC FEVER (uncomplicated) Nutritional Anemia Child Recovered, C/O fever (+) C/O loose stools – 2 episodes C/O Nausea (+) O/E:- Child is Alert / Active HR : 110/min RR : 22/min SpO2 : 98% @ Room Air BP : 100/70 mmHg

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
	O/E child awake, alert, afebrile HR : 102/' O/E PA RR : 24/' SpO2 : 98% @ RA BP : 104/60mmHg CR/PR HT/PR Plan: ① Secure New IV line ② Send CBC repeat. ③ Monitor vitals, w/f shock 4) Continue IV Ceftriaxone 5) If no defervescence by day 5, to add Azithromycin
[redacted] 14/3/26 7:00pm	CC/S/B - [redacted] Δ = ENTERIC FEVER NUTRITIONAL ANEMIA → C/O Loose stools ⊕ 1 episode → C/O fever at 1 am Yesterday (Temp: 100°F) → No C/O Abdominal pain → No C/O Decreased urine output → No C/O Irritability → No C/O Altered Sensorium

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PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
14/3/26 9:45 pm	C/S/B - [redacted]
	Δ = ENTERIC FEVER
	C/o Loose stools 1-episode.
	C/o Fever today at 1:00 Am - 100.4°F
	O/E:- Child is Alert, Active, Afebrile
	PR: 100/min
	RR: 22/min
	SpO2: 98% @ Room Air.
	S/E:-
	P/A: Soft, nontender, Liver palpable 4cm ↓ RCM
	Liver Span: 10.5cm
	RS: B/L AE (+)
	CNS - NFND
	CVS: S1 S2 (+)
	Plan:-
	1) Continue Ceftriaxone
	2) Change Ondansetron to SOS
	3) To Send CBC/CRP in Next Prick
	4) W/F Complications
	[redacted]

[redacted]

[redacted]

Master [redacted] SHEET

2024062570844

Patient's Name ..... PIMS No .....

(9)

Date & Time	
14/8/26 6pm	C/S/B [redacted]
	Δ : Enteric fever.
	(D4) of INJ. CEFTRIAXONE 1.5g BD
	- Voiding well
	- Last fever spike - 101.2F @ 1pm
	O/E : Alert, Active, Afebrile
	Vitals : stable
	U/o (till 6pm) - 7.3 ml/kg/hr (From 8AM)
	S/E : P/A : soft, NT, liver palpable 4 cm b. RCM
	liver span - 10.5 cm
	plan :
	1) Continue IV Antibiotic
	2) W/F Complications
	3) To send CBC, CRP, on next pick
	[redacted]
	[redacted]

[redacted]

[redacted]

2024062570844 SHEET

Patient's Name ..... PIMS No .....

Date & Time	
15/3/26 1:00pm	CLS/B- [redacted]
	Δ- ENTERIC FEVER
	C/o fever spike yesterday @ 10:45pm - 102.5°F
DO Antibiotics ⇓ 5.	C/o Cough (+)
	No C/o Irritability
	No C/o Lethargy
	No C/o Altered Sensorium
⇒ Oral intake improved.	No C/o Abdominal pain
	No C/o Vomiting
	No C/o Head Ache
⇒ No further loose stools	O/E: Alert / Active, Afebrile
	PR: 108/min
⇒ No features of decreased urine output	RR: 22/min
	SpO2: 98% @ Room Air
	S/E:
No c/o headache	P/A: Soft non tender, Liver is palpable 4 ↓ RCM
No c/o vomiting	Liver Span: 10.5cm
	CNS: No focal neurological deficit
No c/o bone pain	CVS: S1S2 (+)
No c/o burning Micturition	RS: B/L AE (+)
No c/o [illegible] hypochondritis.	Plan: [redacted]
No fa[redacted]	1) Continue [redacted]
	2) To Send CBC with PS, CRP in Next Prick.
	3) W/F Complications

NABH/AAC/5d/PROGRESS SHEET/VER-01

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
15/03/06 6.00 pm	D/B [redacted]
	Δ = ENTERIC FEVER
	child reviewed,
	C/o - fever spike @ 12:30pm today - 102.2°F
	No new Complaints
	O/E: Child is Alert, Active, Afebrile
	PR: 100/min
	RR: 22/min
	Spo2 - 98% @ Room Air
	S/E...
	P/A: Soft, non tender, Liver palpable 4cm ↓ RCM
	Liver Span: 10.5 cm
	RS: B/L AE (+)
	CNS - No focal neurological deficit
	CVS - S1S2 (+)
	Plan:-
	1) Continue Ceftriaxone
	2) To Send CBC with PS, CRP in Next Pack.
	3) To Send H1N1 tommorow

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(11)

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
16/3/26 7:00pm	CCS/B – [redacted] (PG) (PG)

Δ = ENTERIC FEVER

C/o Fever Spike @10pm Yesterday – 100.8°F
No C/o Cough
No C/o Nasal discharge
No C/o Altered Sensorium
No C/o Abdominal pain
No C/o Vomiting / No C/o Headache.

O/E:- Alert / Active,
PR : 108/min
RR : 24/min
SpO2 : 98% @ Room Air

S/E:-
P/A :- Soft, non tender, Liver palpable 4cm ↓ RCM
Liver Span : 10.5cm
CNS: No focal neurological deficit
CVS: S1S2 (+)
RS: B/L AE (+)

Plan:-
1) Continue Ceftriaxone
2) To Send CBC with PS, CRP, Widal, Blood
3) To Collect H1N1 Swab Reports

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PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
16/8/24 9:30am	C/S/B - [redacted]
	Child Reviewed,
	C/o fever spike at 10pm yesterday - 100.8°F
	No new Complaints
	O/E: Child is Alert, Active, Afebrile
	PR - 100/min
	RR - 22/min
	SpO2 - 98% @ Room Air
	S/E:-
	P/A: Soft, non tender, Liver is palpable 4cm ↓RCM
	Liver Span - 10.5cm
	RS: B/L AE (+)
	CVS - NFND
	CNS: SLS2 (+)
	Plan:-
	1) Continue Ceftriaxone
	2) To send CBC with PS, CRP in next prick
	3) W/F Complications
[redacted]	[redacted]

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
	O/E: Active, Alert, Afebrile PR - 108/min RR - 22/min SpO2 - 98% @ Room Air
16/3/26 Hb - 9.0 PCV - 28.1 RC - 3.66 Plat - 495 Diff - 9.72 Cons CRP - 8	S/E:- P/A - Soft, non-tender, Liver impalpable & ↓ RCM Liver Span: 8 cm RS - B/L AE (+) CNS - No focal neurological deficit CVS - S1S2 (+) Plan:- 1) Continue Ceftriaxone 2) W/F Complications [redacted]
17/3/26 12:00pM	C/S/B - [redacted] Child Reviewed, C/O Cough (+) No C/O fever spike Last fever spike - yesterday at 12 pm - 100.2°F No new Complaints O/E: child is Active, Alert, Afebrile

[redacted]

[redacted]

(13)

PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time	
	PR - 102/min RR - 22/min SpO2 - 98% @ Room Air GIE :- P/A - Soft, non-tender, Liver is palpable 4 cm ↓ RCM Liver span - 10.5cm RS : BIL AE (+) CNS : No focal neurological deficit CVS : S1S2 (+) Plan :- 1) If no fever spike change to oral antibiotics and discharge tomorrow
[redacted]	
17/3/26 5:30pm	C/S/B - [redacted] Child Reviewed, Child was Afebrile >24hrs O/E: Alert, Active, Afebrile Vitals stable GIE: NAD P/A - Soft, non-tender Liver is palpable 4 cm ↓ RCM Liver span - 10.5cm Plan :- 1) Change to oral antibiotics, if fever spike 2) Plan discharge tomorrow

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name ..... [redacted] ..... PIMS No .....

Date & Time	
18/3/26 9:00pm	C/S/B - [redacted] (PG) [redacted] (PG)

A = ENTERIC FEVER

No C/o fever spike
No C/o Nasal Discharge
No C/o Abdominal pain
No C/o Altered sensorium
No C/o Vomiting
No C/o Headache
No H/o Reduced Urine Output

O/E: Child is Active, Alert, Afebrile
PR - 108/min
RR - 22/min
SpO2 - 98% @ Room Air

S/E:-
P/A: soft, non tender liver is palpable 2 cm below RCM, Liver span of 9 cm
RS: B/L AE (P)
CVS: S1S2 (P)
CNS: NFND

[redacted]

Plan:-
1) To change to oral Antibiotics, fever spike
2) Plan discharge today